Core buildup (D2950) attachment checklist

When it pays, when it bundles, and what proves the difference

D2950 is denied less for lack of need than for lack of separation. Several carriers fold it into the crown fee when the buildup happens in the same appointment as the restoration, so the record has to show a distinct, medically necessary visit.

KNOW THE RULE BEFORE YOU BILL

- ☐ **Same-visit bundling:** many plans bundle D2950 into the crown when both are completed at one appointment. Check the contract line, not the fee schedule.
- ☐ **Separate-visit payment:** plans that pay usually require a different date, a separate anaesthetic entry, and its own charge line.
- ☐ **Post and pin:** D2952 and D2954 have their own bundling rules. Confirm them at the same time you check D2950.
- ☐ **Base and liner:** a medicament, base or liner placed under a restoration is not a core buildup. Do not bill them as one.

DOCUMENT AT THE APPOINTMENT

1. **Tooth and remaining structure.** How much sound tooth is left, and why it will not retain a restoration on its own.
2. **Why a core is needed.** Write "core buildup required to retain the final restoration" — "buildup done" gets bundled.
3. **Endodontic context.** Existing root canal, post space, gutta-percha removal, and the isolation used.
4. **Material and time.** Material placed, the amount, and the appointment length. Separate visit? Note the date.
5. **Radiographs.** Pre-operative image showing the deficiency; post-operative if the payer asks for it.

Answering a bundle denial: send the two dates side by side, the anaesthetic record for each visit, and the note that states retention failed without the core. If the plan genuinely bundles it, stop re-billing and change how the case is scheduled instead — that decision belongs in the fee plan, not in the appeal queue.

BEANbite reads your payer contracts with you.

We map bundling, frequency and attachment rules across your plans, then bill what the contract actually allows.

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